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7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name : **DEVANSH UPADHYAY**
Ref By : **DR. ATUL GATTANI**
Reg No : **HL4001832622 / HYMH/H5603**
Barcode : **HEA093107**
INV : ANTI NUCLEAR ANTIBODY (ANA-IFA)

Age : 23 Year | **Sex :** Male
Registered on : 02-09-2026 10:37 AM
Received on : 02-09-2026 10:51 AM
Reported on : 03-09-2026 12:08 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

ANTI NUCLEAR ANTIBODY (ANA-IFA)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
FLOURESCENCE	NOT SEEN	.	. - NOT SEEN
INTENSITY	NIL	.	. - NIL
TITER	1:80	.	. - 1:80
HEp-2 PATTERN	NIL	.	. - NIL
RESULTS	NEGATIVE	.	. - NEGATIVE
Disease Association	-	.	. - .
ADVISE	-	.	. - .

Interpretation:

A positive result on the **ANA IFA** screen suggests the presence of autoimmune disease, and will reflex to titer and pattern. A low **ANA** titer (1:40 to 1:80) may be associated with preclinical disease or lack of disease. Titers >1:80 are consistent with autoimmune disease.

Clinical significance :

ANA are commonly found in a variety of autoimmune diseases. Antibody frequency increases with age in apparently healthy people. ANA patterns on Hep-2 slides provide only general clues about particles (chromatin, nucleosomes and spliceosomes). ANA patterns (other than centromere pattern) are not reliably correlated with the presence of specific antibodies and must be further evaluated by EIA using individual ENA antigens

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **SONAL MAHAMULKAR**
Ref By : **DR. SELF**
Reg No : **HL7501832422 / MH441**
Barcode : **10826134104**
INV : Anti Thyroglobulin Antibody (Anti-TG)

Age : 63 Year | **Sex :** Female
Registered on : 31-08-2026 04:46 PM
Received on : 01-09-2026 12:18 AM
Reported on : 01-09-2026 03:32 PM

SAMPLE COLLECTED AT :
BIOGENEX DIAGNOSTIC

SAMPLE : Serum

ANTI THYROGLOBULIN ANTIBODY (ANTI-TG)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
ANTI THYROGLOBULIN ANTIBODIES Method: CLIA	45.6	IU/ml	0.00 - 60.00

Clinical significance :
Thyroglobulin (Tg) is a thyroid-specific glycoprotein (approximately 660 KDa) that serves as the source for thyroxine (T4) and triiodothyronine (T3) production within the lumen of thyroid follicles. For T4 and T3 release, Tg is reabsorbed into thyrocytes and proteolytically degraded, liberating T4 and T3 for secretion. Small amounts of intact Tg are secreted alongside T4 and T3 and are detectable in the serum of healthy individuals, with levels roughly paralleling thyroid size (0.5-1.0 ng/mL Tg per gram thyroid tissue, depending on thyroid-stimulating hormone [TSH] level). In situations of disordered thyroid growth (eg, goiter), increased thyroid activity (eg, Graves disease), or glandular destruction (eg, thyroiditis) larger amounts of Tg may be released into the circulation.

Clinically, the main use of serum Tg measurements is in the follow-up of differentiated follicular cell-derived thyroid carcinoma. Because Tg is thyroid-specific, serum Tg concentrations should be undetectable, or very low, after the thyroid gland is removed during treatment for thyroid cancer.

Current clinical guidelines consider a serum Tg of more than1 ng/mL in an athyrotic individual as suspicious of possible residual or recurrent disease. To improve diagnostic accuracy, it is recommended that at least initially this measurement is obtained after TSH stimulation, either following thyroid hormone withdrawal, or after injection of recombinant human TSH. Most patients will have a relatively low risk of recurrence, and will thereafter only require unstimulated Tg measurement. If unstimulated (on thyroxine) serum Tg measurements are less than 0.1 to 0.2 ng/mL, the risk of disease is below 1%. Patients with higher Tg levels, who have no demonstrable remnant of thyroid tissue, might require additional testing, such as further stimulated Tg measurements, neck ultrasound, or isotope imaging. A stimulated Tg above 2 ng/mL is considered suspicious.

The presence of anti-thyroglobulin autoantibodies (TgAb), which occur in 15% to 30% of thyroid cancer patients, could lead to misleading Tg results. In immunometric assays, the presence of TgAb can lead to false low measurement; whereas it might lead to false high results in competitive assays. Traditionally, there have been no reliable means to obtain accurate Tg measurements in patients with TgAb. However, recently, trypsin digestion of serum proteins, which cuts both antibodies and Tg into predictable fragments, has allowed accurate quantification of Tg in samples with antibody interferences through measurement of Tg-specific tryptic peptides by mass

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **SONAL MAHAMULKAR**
Ref By : **DR. SELF**
Reg No : **HL7501832422 / MH441**
Barcode : **10826134104**
INV : Anti Thyroglobulin Antibody (Anti-TG)

Age : 63 Year | **Sex :** Female
Registered on : 31-08-2026 04:46 PM
Received on : 01-09-2026 12:18 AM
Reported on : 01-09-2026 03:32 PM

SAMPLE COLLECTED AT :
BIOGENEX DIAGNOSTIC

SAMPLE : Serum

ANTI THYROGLOBULIN ANTIBODY (ANTI-TG)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
ANTI THYROGLOBULIN ANTIBODIES Method: CLIA	45.6	IU/ml	0.00 - 60.00

Clinical significance :
Thyroglobulin (Tg) is a thyroid-specific glycoprotein (approximately 660 KDa) that serves as the source for thyroxine (T4) and triiodothyronine (T3) production within the lumen of thyroid follicles. For T4 and T3 release, Tg is reabsorbed into thyrocytes and proteolytically degraded, liberating T4 and T3 for secretion. Small amounts of intact Tg are secreted alongside T4 and T3 and are detectable in the serum of healthy individuals, with levels roughly paralleling thyroid size (0.5-1.0 ng/mL Tg per gram thyroid tissue, depending on thyroid-stimulating hormone [TSH] level). In situations of disordered thyroid growth (eg, goiter), increased thyroid activity (eg, Graves disease), or glandular destruction (eg, thyroiditis) larger amounts of Tg may be released into the circulation.

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Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

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REPORT

PT Name	: **SACHIN SAWANT**	**Age :** 39 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. ANGEL PATHOLOGY LAB**	**Registered on** : 29-08-2026 01:53 PM	ANGEL PATHOLOGY LAB BEED 9860018831
Reg No	: **HL7901830185 / MH316**	**Received on** : 30-08-2026 09:06 AM	
Barcode	: **HEB188540**	**Reported on** : 31-08-2026 11:14 AM	
INV	: BIOTIN (VITAMIN B7)		**SAMPLE :** Serum

BIOTIN (VITAMIN B7)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
BIOTIN (VITAMIN B7) Method: Bioassay	1.8	ng/ml	0.2 - 3.0

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
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REPORT

PT Name	: **MANPREET KAUR**	**Age :** 29 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** NEO
Ref By	: **DR. SELF**	**Registered on :** 30-08-2026 12:45 PM	
Reg No	: **HL3301831094 / MH677**	**Received on** : 30-08-2026 05:37 PM	
Barcode	: **HEI057044**	**Reported on** : 31-08-2026 07:28 AM	**SAMPLE :** Serum
INV	: Complement 4 (C4)		

COMPLEMENT 4 (C4)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
COMPLEMENT C4	0.24	G/L	0.10 - 0.40

Method: .

Clinical significance :

The complement system is an integral part of the immune defenses. It can be activated via immune complexes (classic pathway) or by bacterial polysaccharides (alternative pathway). The classic complement pathway consists of recognition, (C1q, C1r, C1s), activation (C2, C3, C4), and attack (C5, C6, C7, C8, C9) mechanisms with respect to their role in antibody-mediated cytolysis. C4 is one of the activation proteins of the classic pathway.

In the absence of C4, immune complexes will not be cleared by C3 activation peptides, but bacterial infections can still be defended via the alternative pathway.

C4 may be decreased in systemic lupus erythematosus, early glomerulonephritis, immune complex disease, cryoglobulinemia, hereditary angioedema, and congenital C4 deficiency.

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **MR. JAYPRAKASH YADAV**	**Age :** 41 Year | **Sex :** Male		**SAMPLE COLLECTED AT :**	
Ref By	: **DR. JITENDRA TEKCHANDANI**	**Registered on**	: 30-08-2026 01:27 PM	NIDHI PATHOLOGY	
Reg No	: **HL6101831172 / MH649**	**Received on**	: 30-08-2026 04:36 PM		
Barcode	: **HEN141634**	**Reported on**	: 30-08-2026 08:00 PM		
INV	: C-PEPTIDE (F)			**SAMPLE :** Serum	

C-PEPTIDE (F)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
C-PEPTIDE (F) Method: CLIA	2.11	ng/mL	0.81 - 3.85

Clinical significance :

C-peptide (connecting peptide), a 31-amino-acid polypeptide, represents the midportion of the proinsulin molecule. Proinsulin resembles a hairpin structure, with an N-terminal and C-terminal, which correspond to the A and B chains of the mature insulin molecule, oriented parallel to each other and linked by disulfide bonds. The looped portion of the hairpin between the A and B chains is called C-peptide. During insulin secretion, C-peptide is enzymatically cleaved off and cosecreted in equimolar proportion with mature insulin molecules.

Following secretion, insulin and C-peptide enter the portal circulation and are routed through the liver where at least 50% of the insulin binds to receptors, initiates specific hepatic actions (stimulation of hepatic glucose uptake and suppression of glycogenolysis, gluconeogenesis, and ketogenesis), and is subsequently degraded. Most of the insulin molecules that pass through the liver into the main circulation bind to peripheral insulin receptors, promoting glucose uptake, while the remaining molecules undergo renal elimination. Unlike insulin, C-peptide is subject to neither hepatic nor significant peripheral degradation but is mainly removed by the kidneys. As a result, C-peptide has a longer half-life than insulin (30-35 minutes versus 5-10 minutes), and the molar ratio of circulating insulin to circulating C-peptide is generally below 1, despite equimolar secretion. Until recently, C-peptide was thought to have no physiological function, but it now appears that there may be specific C-peptide cell-surface receptors (most likely belonging to the super-family of G-protein coupled receptors), which influence endothelial responsiveness and skeletal and renal blood flow.

In most disease conditions associated with abnormal serum insulin levels, the changes in serum C-peptide levels parallel insulin-related alterations (insulin to C-peptide molar ratio < or =1). Both serum C-peptide and serum insulin levels are elevated in renal failure and in disease states that lead to augmented primary endogenous insulin secretion (eg, insulinoma, sulfonylurea intoxication). Both also may be raised in any disease states that cause secondary increases in endogenous insulin secretion mediated through insulin resistance, primarily obesity, glucose intolerance, and early type 2 diabetes mellitus (DM), as well as endocrine disorders associated with hypersecretion of insulin-antagonistic hormones (eg, Cushing syndrome, acromegaly). Failing insulin secretion in type 1 DM and longstanding type 2 DM is associated with corresponding reductions in serum C-peptide levels.

Discordant serum insulin and serum C-peptide abnormalities are mainly observed in 2 situations: exogenous insulin administration and the presence of anti-insulin autoantibodies. Factitious hypoglycemia due to surreptitious insulin administration results in appropriate suppression of endogenous insulin and C-peptide secretion. At the same time, the peripherally administered insulin bypasses the hepatic first-pass metabolism. In these situations, insulin levels are elevated and C-peptide levels are decreased. In patients with insulin antibodies, insulin levels are increased because of the prolonged half-life of autoantibody-bound insulin. Some patients with anti-idiotypic anti-insulin autoantibodies experience episodic hypoglycemia caused by displacement of autoantibody-bound insulin.

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
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REPORT

PT Name : **MRS. SUMAN MHAIDHUNE**
Ref By : **DR. KHER HOSPITAL**
Reg No : **HL6401829122 / MH546**
Barcode : **HEI059311**
INV : C-PEPTIDE (PP)

Age : 68 Year | **Sex :** Female
Registered on : 28-08-2026 12:16 PM
Received on : 28-08-2026 03:06 PM
Reported on : 28-08-2026 08:55 PM

SAMPLE COLLECTED AT :
WE CARE DIAGNOSTIC CENTRE

SAMPLE : Serum

C-PEPTIDE (PP)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
C-PEPTIDE (PP) Method: CLIA	6.31	ng/mL	1.20 - 8.60

Clinical significance :

C-peptide (connecting peptide), a 31-amino-acid polypeptide, represents the midportion of the proinsulin molecule. Proinsulin resembles a hairpin structure, with an N-terminal and C-terminal, which correspond to the A and B chains of the mature insulin molecule, oriented parallel to each other and linked by disulfide bonds. The looped portion of the hairpin between the A and B chains is called C-peptide. During insulin secretion, C-peptide is enzymatically cleaved off and cosecreted in equimolar proportion with mature insulin molecules.

Following secretion, insulin and C-peptide enter the portal circulation and are routed through the liver where at least 50% of the insulin binds to receptors, initiates specific hepatic actions (stimulation of hepatic glucose uptake and suppression of glycogenolysis, gluconeogenesis, and ketogenesis), and is subsequently degraded. Most of the insulin molecules that pass through the liver into the main circulation bind to peripheral insulin receptors, promoting glucose uptake, while the remaining molecules undergo renal elimination. Unlike insulin, C-peptide is subject to neither hepatic nor significant peripheral degradation but is mainly removed by the kidneys. As a result, C-peptide has a longer half-life than insulin (30-35 minutes versus 5-10 minutes), and the molar ratio of circulating insulin to circulating C-peptide is generally below 1, despite equimolar secretion. Until recently, C-peptide was thought to have no physiological function, but it now appears that there may be specific C-peptide cell-surface receptors (most likely belonging to the super-family of G-protein coupled receptors), which influence endothelial responsiveness and skeletal and renal blood flow.

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Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **AYYAN NADAF**
Ref By : **DR. SELF**
Reg No : **HL6501827075 / MH026**
Barcode : **396695434A**
INV : CSF R/M

Age : 17 Year | **Sex :** Male
Registered on : 26-08-2026 12:41 PM
Received on : 26-08-2026 03:06 PM
Reported on : 27-08-2026 05:41 PM

SAMPLE COLLECTED AT :

SAMPLE : Body Fluid

CSF R/M

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
COLOUR Method: .	COLOURLESS	.	. - .
QUANTITY Method: .	05	ML	. - .
SAMPLE TYPE Method: .	CSF	.	. - .
AFB STAIN Method: .	NOT SEEN	.	. - .
OTHERS Method: .	.	.	. - .
POLYMORPHS Method: .	00	%	. - .
LYMPHOCYTES Method: .	100	%	. - .
RBC'S Method: .	NIL	.	. - .
Total Leucocyte Count(TLC) Method: .	314	.	. - .
CHLORIDES	110.2	mmol/L	110 - 135
PROTEINS	0.05	mg/dL	0 - 50
GLUCOSE	98.6	mg/dl	70 - 100
SPECIFIC GRAVITY Method: .	1.020	.	1.020 : EXUDATES
PH	7.0	.	7 - 8
COAGULUM Method: .	ABSENT	.	. - .
APPEARANCE Method: .	SLIGHTLY HAZY	.	. - .
GRAM STAIN Method: .	NO- MICRO ORGANISM SEEN	.	. - .

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **SAMIUN TANMBOLI**	**Age :** 33 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** BIOGENEX DIAGNOSTIC
Ref By	: **DR. SELF**	**Registered on :** 30-08-2026 05:07 PM	
Reg No	: **HL6701831456 / MH441**	**Received on** : 30-08-2026 08:03 PM	
Barcode	: **130826133735**	**Reported on** : 31-08-2026 07:08 AM	**SAMPLE :** STOOL
INV	: HELICOBACTER PYLORI ANTIGEN (H. PYLORI), STOOL		

HELICOBACTER PYLORI ANTIGEN (H. PYLORI), STOOL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HELICOBACTER PYLORI ANTIGEN, Method: EIA	0.27	.	0.0 - 0.9

NOTE-

1. False negative results may be obtained within 2 weeks of treatment with antimicrobials, bismuth, or proton pump inhibitors. In such cases it is recommended to repeat the test 2 weeks after discontinuing therapy.
2. A positive result indicates the presence of *H. pylori* antigens. A negative result indicates the absence of detectable antigen but does not eliminate the possibility of infection due to *Helicobacter pylori.*
3. Test results should be used in conjunction with clinical findings and results of other diagnostic procedures.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **MRS KALPANA K BHOYE**	**Age :** 37 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. YASHRAJ LAB**	**Registered on :** 28-08-2026 11:19 PM	BHUSHAN BANKAR
Reg No	: **HL6001829730 / MH004/008**	**Received on** : 05-09-2026 08:53 PM	
Barcode	: **HEJ088966**	**Reported on** : 11-09-2026 07:27 PM	
INV	: Histopathology (L)		**SAMPLE :** Tissue

HISTOPATHOLOGY (L)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

BIOPSY-15040/26

NOS- UTERINE MASS FOR HPE.

GROSS- RECEIVED AN UTERUS WITH CERVIX MEASURING 18X15X10 CM , ALONG WITH BILATERAL ADNEXAE AND TUBE , ONE OF THE ADNEXAE AND TUBE MEASURING 3X2 CM AND ANOTHER ADNEXAE AND TUBE MEASURING 2X2 CM. OUTERSURFACE UNREMARKABLE CUT SURFACE ENDOCERVICAL CANAL MEASURES 2 CM,ENDOMETRIAL CAVITY MEASURES 3 CM, AVERAGE MYOMETRIAL THICKNESS MEASURES 3 CM.ONE BIT FROM CERVIX GIVEN IN ONE BLOCK A,ONE BIT FROM ENDOMETRIUM IN ONE BLOCK B. WHORLED AREAS IDENTIFIED MEASURING 12X12 CM ,ONE BIT GIVEN IN ONE BLOCK C.FROM ONE OF THE OVARY AND FALLAPION TUBE ONE BIT GIVEN IN BLOCK D.FROM ANOTHER OVARY AND FALLAPION TUBE ,ONE BIT GIVEN IN ONE BLOCK E.

MICROSCOPIC PICTURE--A-- SECTION STUDIED FROM CERVIX SHOWS POLYPOID LESIONS WITH SUPERFICIAL
LOCATION SHOW COMPLEX, BACK-TO-BACK TUBULAR AND CYSTICALLY DILATED GLANDS WITH MINIMAL STROMA. GLANDS CONTAIN MUCIN AND ACUTE INFLAMMATION, SET IN A MIXED STROMAL INFLAMMATORY BACKGROUND. LINING EPITHELIUM IS BLAND (CUBOIDAL, COLUMNAR, OR FLATTENED)
WITH VARIABLE SUBNUCLEAR/SUPRANUCLEAR VACUOLES. RESERVE OR IMMATURE SQUAMOUS CELLS
ARE PRESENT BENEATH, WITH AREAS OF IMMATURE OR MATURE SQUAMOUS METAPLASIA.

IMPRESSION- SUGGESTIVE OF PAPILLARY ENDOCERVICITIS WITH MICRO GLANDULAR HYPERPLASIA.

MICROSCOPIC PICTURE-B- SECTION STUDIED SHOWS ENDOMETRIAL TISSUE WITH ENDOMETRIAL GLANDS AND STROMA,WITH GLAND TO STROMA RATIO IS 2:1 WITH INCREASED GLAND TO STROMA PROLIFERATION,STROMAL INFLAMMATORY INFILTRATES.

IMPRESSION- SUGGESTIVE OF COMPLEX ENDOMETRIAL HYPERPLASIA WITHOUT ATYPIA.

MICROSCOPIC PICTURE-C- SECTION S SHOWS CONVENTIONAL LEIOMYOMA (SPINDLE): WELL DEFINED BORDERS NORMO CELLULAR INTERSECTING FASCICLES OF MONOTONOUS SPINDLE CELLS WITH INDISTINCT BORDERS, EOSINOPHILIC CYTOPLASM, CIGAR SHAPED NUCLEI (WITH TAPERED ENDS) AND SMALL NUCLEOLI BLOOD VESSELS WITH THICK WALLS.

IMPRESSION- SUGGESTIVE OF LEIOMYOMA .

MICROSCOPIC PICTURE-D,E -SECTION STUDIED FROM OVARY SHOWS MULTILOCULAR CYSTS LINED BY A SINGLE LAYER OF TALL, COLUMNAR ,CILIATED CELLS .STROMA CONTAINS

SPINDLE FIBROBLASTS,PAPILLAE,GLANDS AND CYSTS FORMING A PAPILLAE.

IMPRESSION- SUGGESTIVE OF SIMPLE OVARIAN CYST.

MICROSCOPIC PICTURE-SECTION STUDIED SHOWS SHORT THICK PLICAE WITH FIBROUS STROMA,LYMPHOCYTES,PLASMA CELLS EPITHELIAL HYPERPLASIA NECROTISING GRANULOMAS.

IMPRESSION- SUGGESTIVE OF CHRONIC SALPHINGITIS.

~~End of report~~

Dr. NAMARATA SHARAN (MD PATH)
Consultant Pathologist (Reg. 41002)

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7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **RAJESH MAHALE.**	**Age :** 58 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF -**	**Registered on** : 21-08-2026 11:33 PM	ANNAPURNA LAB DHULE 9823099808
Reg No	: **HL9401822169 / MH103**	**Received on** : 22-08-2026 08:52 AM	
Barcode	: **HEN125516**	**Reported on** : 24-08-2026 12:41 PM	
INV	: COMPLETE VITAMIN PROFILE		**SAMPLE :** Serum

NIACIN (VITAMIN B3)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
VITAMIN B3 Method: Quantitative High Performance Liquid	2.51	ug/mL	0.5 - 8.9

~~End of report~~

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REPORT

PT Name	: **RAJESH MAHALE.**	**Age :** 58 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF -**	**Registered on** : 21-08-2026 11:33 PM	ANNAPURNA LAB DHULE 9823099808
Reg No	: **HL9401822169 / MH103**	**Received on** : 22-08-2026 08:52 AM	
Barcode	: **HEN125516**	**Reported on** : 24-08-2026 12:41 PM	**SAMPLE :** Serum
INV	: COMPLETE VITAMIN PROFILE		

PANTOTHENIC ACID (VITAMIN B5)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PANTOTHENIC ACID Method: Quantitative Cell	1.95	mcmol/L	1.6 - 2.7

~~End of report~~

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REPORT

PT Name	: **RAISA ANSARI**	**Age :** 47 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. UZAIR MUQRI**	**Registered on :** 28-08-2026 04:24 PM	ABDUL REHMAN
Reg No	: **HL9301829471 / RAJ41**	**Received on** : 28-08-2026 06:32 PM	
Barcode	: **HEL002484**	**Reported on** : 29-08-2026 08:48 AM	
INV	: Pyridoxal 5 Phosphate (VITAMIN B6)		**SAMPLE :** Serum

PYRIDOXAL 5 PHOSPHATE (VITAMIN B6)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Pyridoxal 5 phosphate (VITAMIN B6) Method: Quantitative High Performance Liquid Chromatography	87.3	nmol/L	20.0 - 125.0

INTERPRETATION

Pyridoxal 5'-phosphate measured in a specimen collected following an 8 hour or overnight fast accurately indicates vitamin B_6 nutritional status. Non-fasting specimen concentration reflects recent vitamin intake.

This test was developed and its performance characteristics determined by ARUP Laboratories. It has not been cleared or approved by the US Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes.

~~End of report~~

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REPORT

PT Name	: **RAJESH MAHALE.**	**Age :** 58 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF -**	**Registered on** : 21-08-2026 11:33 PM	ANNAPURNA LAB DHULE 9823099808
Reg No	: **HL9401822169 / MH103**	**Received on** : 22-08-2026 08:52 AM	
Barcode	: **HEN125516**	**Reported on** : 24-08-2026 12:41 PM	
INV	: COMPLETE VITAMIN PROFILE		**SAMPLE :** Serum

RIBOFLAVIN (VITAMIN B2)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Riboflavin (VITAMIN B2) Method: Quantitative High Performance Liquid Chromatography	25.3	nmol/L	5.0 - 50.0

~~End of report~~

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REPORT

PT Name	: **MRS PRIYANKA ARKADE**	**Age :** 29 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** RUPESH DIGNOSTIC
Ref By	: **DR. SELF**	**Registered on :** 31-08-2026 08:00 PM	
Reg No	: **HL4901832562 / MH601**	**Received on** : 01-09-2026 06:40 PM	
Barcode	: **HEI033681**	**Reported on** : 02-09-2026 10:43 AM	
INV	: Thiamine (VITAMIN B 1)		**SAMPLE :** Serum

THIAMINE (VITAMIN B 1)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Thiamine (VITAMIN B 1) Method: Quantitative High Performance Liquid Chromatography	9.5	nmol/L	4.0 - 15.0

INTERPRETATION

Thiamine (vitamin B1) is reported. However, thiamine diphosphate (TDP), the biologically active form of thiamine, is not found in measurable concentrations in plasma, and is best determined in whole blood specimens. Plasma thiamine concentration reflects recent intake rather than body stores.

This test was developed and its performance characteristics determined by ARUP Laboratories. It has not been cleared or approved by the US Food and Drug Administration. This test was performed in a CLIA certified laboratory and is intended for clinical purposes

~~End of report~~

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REPORT

PT Name : **RAJESH MAHALE.**
Ref By : **DR. SELF -**
Reg No : **HL9401822169 / MH103**
Barcode : **HEN125516**
INV : VITAMIN C

Age : 58 Year | **Sex :** Male
Registered on : 21-08-2026 11:33 PM
Received on : 22-08-2026 08:52 AM
Reported on : 24-08-2026 12:42 PM

SAMPLE COLLECTED AT :
ANNAPURNA LAB DHULE 9823099808

SAMPLE : Serum

VITAMIN C

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
VITAMIN-C Method: Liquid Chromatography-Tandem Mass Spectrometry	1.02	mg/dL	0.4 - 2.0

Clinical significance :

Vitamin C, also known as L-ascorbic acid or simply ascorbic acid, is a water-soluble vitamin that is naturally present in some foods, added to others, and available as a dietary supplement. Humans, unlike most animals, are unable to synthesize vitamin C endogenously, so it is an essential dietary component. Vitamin C is required for the enzymatic amidation of neuropeptides, production of adrenal cortical steroid hormones, promotion of the conversion of tropocollagen to collagen, and metabolism of tyrosine and folate. It also plays a role in lipid and vitamin metabolism and is a powerful reducing agent or antioxidant. Specific actions include: activation of detoxifying enzymes in the liver, antioxidation, interception and destruction of free radicals, preservation and restoration of the antioxidant potential of vitamin E, and blockage of the formation of carcinogenic nitrosamines. In addition, vitamin C appears to function in a variety of other metabolic processes in which its role has not been well characterized.

Prolonged deficiency of vitamin C leads to the development of scurvy, a disease characterized by an inability to form adequate intercellular substance in connective tissues. This results in the formation of swollen, ulcerative lesions in the gums, mouth, and other tissues that are structurally weakened. Early symptoms may include weakness, easy fatigue and listlessness, as well as shortness of breath, and aching joints, bones, and muscles.

The need for vitamin C can be increased by the use of aspirin, oral contraceptives, tetracycline, and a variety of other medications. Psychological stress and advancing age also tend to increase the need for vitamin C. Among the elderly, lack of fresh fruit and vegetables often adds vitamin C depletion to the inherently increased need, with development of near-scurvy status

Please correlate with clinical conditions.

~~End of report~~

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